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THREE YEAR RESULTS OF ENUCLEATION SURGERY WITH OR WITHOUT PREOPERATIVE RADIATION THERAPY (PERT) IN THE PROSPECTIVE RANDOMIZED COLLABORATIVE OCULAR MELANOMA STUDY (COMS) LARGE TUMOR TRIAL

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Purpose: To describe the three year results of enucleation surgery following standard enucleation and standard enucleation preceded by preoperative radiation in the COMS.

Materials & Methods: The COMS is a prospective randomized multicenter controlled clinical trial. Central Units are charged with carefully defined quality assurance. 1003 patients with large choroidal melanoma were seen at over 40 COMS centers across North America. Members of these centers are required to pass initial and periodic subsequent certification procedures. Patients were randomized to enucleation versus enucleation preceded by 20 Gy external beam radiation followed by enucleation. Enucleation alone was performed in 503 patients, 491 patients received PERT, and 9 patients were not treated as assigned. All patients have been enrolled for two years or longer and 89% for three years or longer.

Results: This report summarizes the three year local complications and local recurrences after enucleation surgery with or without PERT. The short term complications of enucleation surgery in this cohort have been previously described. Incidence and prevalence rates will be reported for the following local complications (cumulative incidence over all follow up time).

	Enucleation Number (%)	PERT Number (%)
Poor prosthetic mobility	112 (22)	116 (23)
Poor alignment of prosthesis	74 (15)	69 (14)
Ptosis	45 (9)	19 (4)
Loss of eye lashes	6 (1)	15 (3)
Dislocation or implant expulsion	12 (2)	15 (3)
Eyelid ulceration	5 (1)	6 (1)
Tumor recurrence in orbit	4 (0.8)	1 (0.2)

Conclusion: Ptosis was reported in nearly twice as many patients treated with enucleation alone as in patients who received PERT. Tumor recurrence in the orbit was seen in 4 (0.8%) of patients treated with enucleation alone but in only one patient (0.2%) who received PERT. No treatment recommendation will be made by the COMS until mortality data become available.

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MUCOSAL MELANOMA OF THE HEAD AND NECK

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Purpose: 1) To determine the incidence of regional recurrence in patients with mucosal melanomas of the head and neck who receive no elective neck treatment 2) To quantitate the incidence of distant metastases 3) To assess whether radiation therapy can improve the local control outcome in patients with mucosal melanoma

Materials & Methods: Between 1961-1995, 40 previously untreated patients with mucosal melanoma of the head and neck were treated with surgery +/- radiotherapy at M. D. Anderson Cancer Center. The median age was 62 years (range, 23-82). There were 14 females and 26 males. Primary tumor sites were: oral cavity (OC)-16, nasal cavity and paranasal sinus (NC/PNS)- 22, and pharynx-2. Staging, using the AJCC classification for epithelial carcinomas, was as follows: T1-21, T2-8, T3-5, T4-5, TX-1. Thirty-seven patients presented without clinical adenopathy; 3 patients had N2b disease. Treatment of the primary tumor included surgery alone (S) -20 and surgery and postoperative radiotherapy (S+X) - 20. Only 3 patients had a neck dissection. 90% of the S patients were stage T1-2 versus 55% of the S+X patients. Eight patients received 30 Gy in 5 fractions, 11 were given 60 Gy in 30 fractions, and one patient received 72 Gy in 36 fractions.

Results: The table shows the 5-yr. actuarial rates of local control, distant metastases, disease-free survival, and overall survival for all patients, and also stratified by treatment and site of disease. Patients who underwent S + X had a higher 5-year local control rate and disease-free survival than patients who were treated with surgery alone (p = ns). The 5-year local control rate for patients with NC/PNS was 43%, compared to 84% for OC patients (p= 0.08). For 33 patients who had no clinical adenopathy and who did not receive elective neck treatment, the 5-year actuarial regional failure rate was 39%; the 5-year regional failure rate for OC patients and NC/PNS patients was 58% and 28%, respectively. For patients with early stage (T1+T2) primary tumors who received no elective neck treatment, the 5-year regional failure rate was 36%.

	All pts. (n=40)	S (n=20)	S + X (n=20)	OC (n=16)	NC/PNS (n= 22)	Pharynx (n= 2)
Local Control	65%	59%	73%	84%	43%	2/2
Distant Metastasis	60%	63%	67%	60%	60%	1/2
Disease-free Surv.	17%	7%	25%	19%	6%	1/2
Overall Survival	36%	38%	33%	44%	26%	1/2

Conclusions: Despite having more advanced staged primary tumors, patients with mucosal melanoma who received radiotherapy had a higher local control rate than patients who were treated with surgery alone. Therefore, we recommend that all patients with NC/PNS melanomas, and patients with unfavorable OC primary tumors (>4 cm or close or positive margins) receive postoperative radiotherapy. Patients with mucosal melanoma have a high incidence of regional failure and should undergo elective neck treatment. The high incidence of distant metastases warrants assessment of systemic therapy in patients with mucosal melanoma.